



US012403256B2

(12) **United States Patent**
Rosinko

(10) **Patent No.:** **US 12,403,256 B2**
(45) **Date of Patent:** ***Sep. 2, 2025**

(54) **SIMPLIFIED INSULIN DELIVERY FOR DIABETICS**

(71) Applicant: **Tandem Diabetes Care, Inc.**, San Diego, CA (US)

(72) Inventor: **Mike Rosinko**, Anaheim, CA (US)

(73) Assignee: **Tandem Diabetes Care, Inc.**, San Diego, CA (US)

(*) Notice: Subject to any disclaimer, the term of this patent is extended or adjusted under 35 U.S.C. 154(b) by 638 days.

This patent is subject to a terminal disclaimer.

(21) Appl. No.: **17/825,327**

(22) Filed: **May 26, 2022**

(65) **Prior Publication Data**

US 2022/0280719 A1 Sep. 8, 2022

Related U.S. Application Data

(63) Continuation of application No. 16/269,767, filed on Feb. 7, 2019, now Pat. No. 11,364,340, which is a continuation of application No. 13/800,595, filed on Mar. 13, 2013, now Pat. No. 10,201,656.

(51) **Int. Cl.**

A61M 5/172 (2006.01)

A61M 5/142 (2006.01)

(52) **U.S. Cl.**

CPC **A61M 5/172** (2013.01); **A61M 5/14244** (2013.01); **A61M 5/1723** (2013.01); **A61M 2005/14208** (2013.01); **A61M 2205/18** (2013.01); **A61M 2205/505** (2013.01)

(58) **Field of Classification Search**

CPC **A61M 5/172**; **A61M 5/14244**; **A61M 5/1723**; **A61M 2005/14208**; **A61M 2205/18**; **A61M 2205/505**

See application file for complete search history.

(56) **References Cited**

U.S. PATENT DOCUMENTS

2,462,596 A	2/1949	Bent
2,629,376 A	2/1953	Pierre et al.
2,691,542 A	10/1954	Chenoweth
3,059,639 A	10/1962	Blackman et al.
4,469,481 A	9/1984	Kobayashi

(Continued)

FOREIGN PATENT DOCUMENTS

DE	399065 C	7/1924
DE	18919407 A1	11/1999

(Continued)

OTHER PUBLICATIONS

Application and File History for U.S. Appl. No. 13/800,595, filed Mar. 13, 2013, inventors Saint.

(Continued)

Primary Examiner — Theodore J Stigell

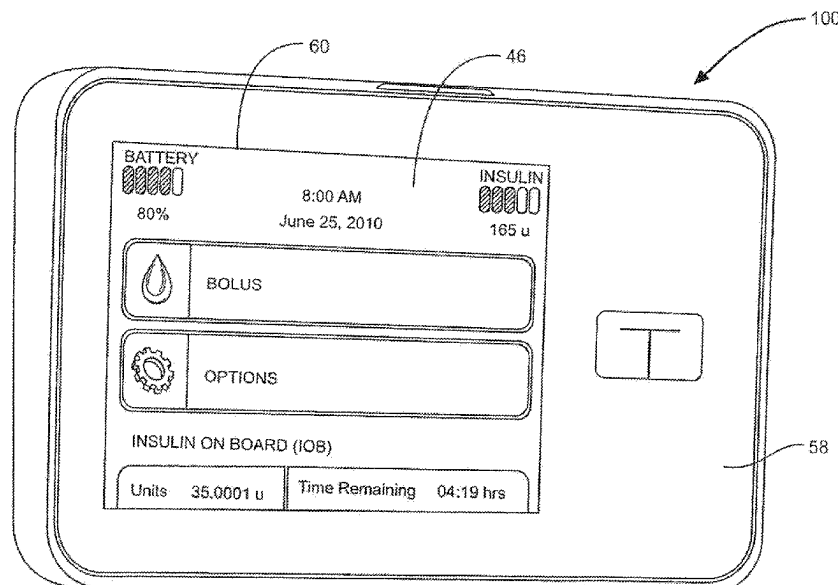
(74) *Attorney, Agent, or Firm* — Merchant & Gould P.C.

(57)

ABSTRACT

A simplified insulin pump allows diabetics to identify routine patterns in their daily lifestyle that provide a generally accurate typical pattern of activity and meals. An infusion regimen can be programmed into the pump that generally correlates with this typical pattern to provide a viable treatment option for a diabetic without the need for specific and precise matching of insulin to carbohydrate consumption or activity level.

16 Claims, 4 Drawing Sheets



(56)

References Cited

U.S. PATENT DOCUMENTS

4,790,829	A *	12/1988	Bowden	A61M 39/14 604/533	8,133,197	B2	3/2012	Blomquist et al.
5,395,326	A	3/1995	Haber et al.		8,152,789	B2	4/2012	Starkweather et al.
5,919,216	A	7/1999	Houben et al.		8,206,350	B2	6/2012	Mann et al.
6,034,054	A	3/2000	Defelippis et al.		8,208,984	B2	6/2012	Blomquist et al.
6,471,689	B1	10/2002	Joseph et al.		8,219,222	B2	7/2012	Blomquist
6,544,212	B2	4/2003	Galley et al.		8,221,345	B2	7/2012	Blomquist
6,551,276	B1	4/2003	Mann et al.		8,250,483	B2	8/2012	Blomquist
6,554,798	B1	4/2003	Mann et al.		8,257,300	B2	9/2012	Budiman et al.
6,558,351	B1	5/2003	Steil et al.		8,287,487	B2	10/2012	Estes
6,562,001	B2	5/2003	Lebel et al.		8,287,495	B2	10/2012	Michaud et al.
6,571,128	B2	5/2003	Lebel et al.		8,298,184	B2	10/2012	Diperna et al.
6,572,542	B1	6/2003	Houben et al.		8,311,749	B2	11/2012	Brauker et al.
6,577,899	B2	6/2003	Lebel et al.		8,337,469	B2	12/2012	Eberhart et al.
6,648,821	B2	11/2003	Lebel et al.		8,344,847	B2	1/2013	Moberg
6,668,196	B1	12/2003	Villegas et al.		8,346,399	B2	1/2013	Kanderian, Jr. et al.
6,740,072	B2	5/2004	Starkweather et al.		8,348,923	B2	1/2013	Kanderian, Jr. et al.
6,740,075	B2	5/2004	Lebel et al.		8,414,523	B2	4/2013	Blomquist et al.
6,744,350	B2	6/2004	Blomquist		8,454,510	B2	6/2013	Yodfat et al.
6,810,290	B2	10/2004	Lebel et al.		8,454,576	B2	6/2013	Mastrototaro et al.
6,827,702	B2	12/2004	Lebel et al.		8,454,581	B2	6/2013	Estes et al.
6,852,104	B2	2/2005	Blomquist		8,465,460	B2	6/2013	Yodfat et al.
6,862,466	B2	3/2005	Ackerman		8,573,027	B2	11/2013	Rosinko et al.
6,872,200	B2	3/2005	Mann et al.		8,657,779	B2	2/2014	Blomquist
6,873,268	B2	3/2005	Lebel et al.		8,712,748	B2	4/2014	Thukral
6,906,028	B2	6/2005	Defelippis et al.		8,753,316	B2	6/2014	Blomquist
6,936,029	B2	8/2005	Mann et al.		8,775,877	B2	7/2014	Mcvey et al.
6,958,705	B2	10/2005	Lebel et al.		8,882,701	B2	11/2014	Debelser et al.
6,974,437	B2	12/2005	Lebel et al.		8,938,306	B2	1/2015	Lebel
6,979,326	B2	12/2005	Mann et al.		8,986,253	B2	3/2015	Diperna
6,997,920	B2	2/2006	Mann et al.		8,992,475	B2	3/2015	Mann
6,999,854	B2	2/2006	Roth		9,008,803	B2	4/2015	Blomquist
7,025,743	B2	4/2006	Mann et al.		9,114,210	B2	8/2015	Estes
7,033,338	B2	4/2006	Milks et al.		9,474,856	B2	10/2016	Blomquist
7,041,082	B2	5/2006	Blomquist et al.		9,492,608	B2	11/2016	Saint
7,109,878	B2	9/2006	Mann et al.		9,603,995	B2	3/2017	Rosinko et al.
7,204,823	B2	4/2007	Estes et al.		9,737,656	B2	8/2017	Blomquist
7,258,864	B2	8/2007	Clark		9,940,441	B2	4/2018	Walsh
7,267,665	B2	9/2007	Steil et al.		10,016,561	B2	7/2018	Saint et al.
7,354,420	B2	4/2008	Steil et al.		10,049,768	B2	8/2018	Blomquist
7,402,153	B2	7/2008	Steil et al.		10,201,656	B2 *	2/2019	Rosinko A61M 5/14244
7,442,186	B2	10/2008	Blomquist		11,364,340	B2 *	6/2022	Rosinko A61M 5/1723
7,497,827	B2	3/2009	Brister et al.		2002/0016568	A1	2/2002	Lebel
7,510,544	B2	3/2009	Vilks et al.		2002/0019606	A1	2/2002	Lebel
7,515,060	B2	4/2009	Blomquist		2002/0065454	A1	5/2002	Lebel
7,517,530	B2	4/2009	Clark		2002/0107476	A1	8/2002	Mann
7,547,281	B2	6/2009	Hayes et al.		2003/0055406	A1	3/2003	Lebel
7,569,030	B2	8/2009	Lebel et al.		2003/0065308	A1	4/2003	Lebel et al.
7,572,789	B2	8/2009	Cowen et al.		2003/0163088	A1	8/2003	Blomquist
7,642,232	B2	1/2010	Green et al.		2003/0163223	A1	8/2003	Blomquist
7,651,489	B2	1/2010	Estes et al.		2003/0163789	A1	8/2003	Blomquist
7,674,485	B2	3/2010	Bhaskaran et al.		2003/0212364	A1	11/2003	Mann et al.
7,678,071	B2	3/2010	Lebel et al.		2004/0193090	A1	9/2004	Lebel et al.
7,678,762	B2	3/2010	Green et al.		2005/0022274	A1	1/2005	Campbell et al.
7,678,763	B2	3/2010	Green et al.		2005/0143864	A1	6/2005	Blomquist
7,704,226	B2	4/2010	Mueller, Jr. et al.		2005/0171513	A1	8/2005	Mann et al.
7,711,402	B2	5/2010	Shults et al.		2005/0272640	A1	12/2005	Doyle
7,734,323	B2	6/2010	Blomquist et al.		2005/0273059	A1 *	12/2005	Mernoe A61M 5/158 604/131
7,751,907	B2	7/2010	Blomquist		2006/0173406	A1	8/2006	Hayes
7,756,722	B2	7/2010	Levine et al.		2006/0173444	A1	8/2006	Choy et al.
7,766,831	B2	8/2010	Dipierro et al.		2007/0016170	A1	1/2007	Kovelman
7,785,313	B2	8/2010	Mastrototaro		2007/0033074	A1	2/2007	Nitzan et al.
7,806,886	B2	10/2010	Kanderian, Jr. et al.		2007/0066956	A1	3/2007	Finkel
7,815,602	B2	10/2010	Mann et al.		2007/0112298	A1	5/2007	Mueller et al.
7,819,843	B2	10/2010	Mann et al.		2007/0118405	A1	5/2007	Campbell et al.
7,831,310	B2	11/2010	Lebel et al.		2007/0161955	A1	7/2007	Bynum et al.
7,837,647	B2	11/2010	Estes et al.		2007/0287985	A1	12/2007	Estes et al.
7,850,641	B2	12/2010	Lebel et al.		2008/0033357	A1	2/2008	Mann et al.
7,905,859	B2	3/2011	Bynum et al.		2008/0071580	A1	3/2008	Marcus et al.
7,920,907	B2	4/2011	McGarraugh et al.		2008/0097289	A1	4/2008	Steil et al.
7,976,870	B2	7/2011	Berner et al.		2008/0114299	A1	5/2008	Damgaard-Sorensen
8,012,119	B2	9/2011	Estes et al.		2008/0147004	A1	6/2008	Mann et al.
8,062,249	B2	11/2011	Wilinska et al.		2008/0147050	A1	6/2008	Mann et al.
8,105,268	B2	1/2012	Lebel et al.		2008/0171697	A1	7/2008	Jacotot et al.
8,119,593	B2	2/2012	Richardson et al.		2008/0171967	A1	7/2008	Blomquist et al.
					2008/0172026	A1	7/2008	Blomquist
					2008/0172027	A1	7/2008	Blomquist
					2008/0172028	A1	7/2008	Blomquist
					2008/0172029	A1	7/2008	Blomquist

(56)

References Cited

U.S. PATENT DOCUMENTS

2008/0172030	A1	7/2008	Blomquist	2011/0144586	A1	6/2011	Michaud et al.
2008/0172031	A1	7/2008	Blomquist	2011/0144616	A1	6/2011	Michaud et al.
2008/0177165	A1	7/2008	Blomquist et al.	2011/0152770	A1	6/2011	Diperna et al.
2008/0183060	A1	7/2008	Steil et al.	2011/0152824	A1	6/2011	Diperna et al.
2008/0269714	A1	10/2008	Mastrototaro et al.	2011/0160695	A1	6/2011	Sigrist et al.
2008/0269723	A1	10/2008	Mastrototaro et al.	2011/0166544	A1	7/2011	Verhoef et al.
2008/0300534	A1	12/2008	Blomquist	2011/0184264	A1	7/2011	Galasso et al.
2009/0006129	A1	1/2009	Thukral	2011/0196213	A1	8/2011	Thukral et al.
2009/0036752	A1	2/2009	Deadwyler et al.	2011/0256024	A1	10/2011	Cole et al.
2009/0163855	A1	6/2009	Shin et al.	2012/0013625	A1	1/2012	Blomquist et al.
2009/0177142	A1	7/2009	Blomquist et al.	2012/0013802	A1	1/2012	Blomquist et al.
2009/0177147	A1	7/2009	Blomquist et al.	2012/0029433	A1	2/2012	Michaud et al.
2009/0247931	A1	10/2009	Damgaard-Sorensen	2012/0029468	A1	2/2012	Diperna
2009/0275886	A1	11/2009	Blomquist et al.	2012/0030610	A1	2/2012	Diperna et al.
2010/0008795	A1	1/2010	Diperna	2012/0041415	A1	2/2012	Estes et al.
2010/0094251	A1	4/2010	Estes	2012/0059353	A1	3/2012	Kovatchev et al.
2010/0114015	A1	5/2010	Kanderian, Jr et al.	2012/0220939	A1	8/2012	Yodfat
2010/0125241	A1	5/2010	Prud et al.	2012/0226124	A1	9/2012	Blomquist
2010/0145262	A1	6/2010	Bengtsson et al.	2012/0232484	A1	9/2012	Blomquist
2010/0145303	A1*	6/2010	Yodfat A61M 5/14244 604/151	2012/0232485	A1	9/2012	Blomquist
2010/0160740	A1	6/2010	Cohen et al.	2012/0232520	A1	9/2012	Sloan et al.
2010/0161236	A1	6/2010	Cohen et al.	2012/0232521	A1	9/2012	Blomquist
2010/0161346	A1	6/2010	Getschmann et al.	2012/0238854	A1	9/2012	Blomquist
2010/0174266	A1	7/2010	Estes	2012/0265722	A1	10/2012	Blomquist
2010/0174553	A1	7/2010	Kaufman et al.	2012/0277667	A1	11/2012	Yodfat et al.
2010/0185142	A1	7/2010	Kamen et al.	2012/0302991	A1	11/2012	Blomquist
2010/0185175	A1	7/2010	Kamen et al.	2012/0330227	A1	12/2012	Budiman et al.
2010/0198142	A1	8/2010	Sloan et al.	2013/0046281	A1	2/2013	Javitt
2010/0198520	A1	8/2010	Breton et al.	2013/0053816	A1	2/2013	Diperna et al.
2010/0218132	A1	8/2010	Soni et al.	2013/0131630	A1	5/2013	Blomquist
2010/0249530	A1	9/2010	Rankers et al.	2014/0276420	A1	9/2014	Rosinko
2010/0249561	A1	9/2010	Patek et al.	2014/0276531	A1	9/2014	Walsh
2010/0262117	A1	10/2010	Magni et al.	2014/0276553	A1	9/2014	Rosinko et al.
2010/0262434	A1	10/2010	Shaya	2014/0276556	A1	9/2014	Saint et al.
2010/0274592	A1	10/2010	Nitzan et al.	2014/0276574	A1	9/2014	Saint
2010/0280329	A1	11/2010	Randloev et al.	2015/0182693	A1	7/2015	Rosinko
2010/0286653	A1	11/2010	Kubel et al.	2015/0182695	A1	7/2015	Rosinko
2010/0292634	A1	11/2010	Kircher, Jr. et al.	2015/0217044	A1	8/2015	Blomquist
2010/0305545	A1	12/2010	Kanderian, Jr. et al.	2015/0314062	A1	11/2015	Blomquist et al.
2010/0324382	A1	12/2010	Cantwell et al.	2016/0082188	A1	3/2016	Blomquist et al.
2011/0006876	A1	1/2011	Moberg et al.	2017/0165416	A1	6/2017	Saint
2011/0009725	A1	1/2011	Hill et al.	2017/0246380	A1	8/2017	Rosinko et al.
2011/0009813	A1	1/2011	Rankers	2017/0312423	A1	11/2017	Rosinko
2011/0021898	A1	1/2011	Wei et al.	2018/0133397	A1	5/2018	Estes
2011/0033833	A1	2/2011	Blomquist et al.	2018/0133398	A1	5/2018	Blomquist
2011/0040251	A1	2/2011	Blomquist et al.	2018/0137252	A1	5/2018	Mairs
2011/0054390	A1	3/2011	Searle et al.	2018/0226145	A1	8/2018	Walsh
2011/0054391	A1	3/2011	Ward et al.	2018/0233221	A1	8/2018	Blomquist
2011/0071464	A1	3/2011	Palerm	2018/0361060	A9	12/2018	Rosinko
2011/0071765	A1	3/2011	Yodfat et al.				
2011/0098637	A1	4/2011	Hill				
2011/0098638	A1	4/2011	Chawla et al.				
2011/0098674	A1	4/2011	Vicente et al.				
2011/0106011	A1	5/2011	Cinar et al.				
2011/0106050	A1	5/2011	Yodfat et al.				
2011/0112505	A1	5/2011	Starkweather et al.				
2011/0112506	A1	5/2011	Starkweather et al.				
2011/0118578	A1	5/2011	Timmerman				
2011/0124996	A1	5/2011	Rienke et al.				
2011/0125095	A1	5/2011	Lebel et al.				
2011/0130746	A1	6/2011	Budiman				
2011/0137239	A1	6/2011	Debelser et al.				

FOREIGN PATENT DOCUMENTS

WO	03082091	A2	10/2003
WO	2007065944	A1	6/2007
WO	2009016636	A2	2/2009
WO	2013184896	A1	12/2013

OTHER PUBLICATIONS

International Preliminary Report on Patentability for Application No. PCT/US2014/021075, mailed Sep. 24, 2015, 12 pages.
 International Search Report for Application No. PCT/US2014/021075, mailed Jul. 1, 2014, 24 pages.

* cited by examiner

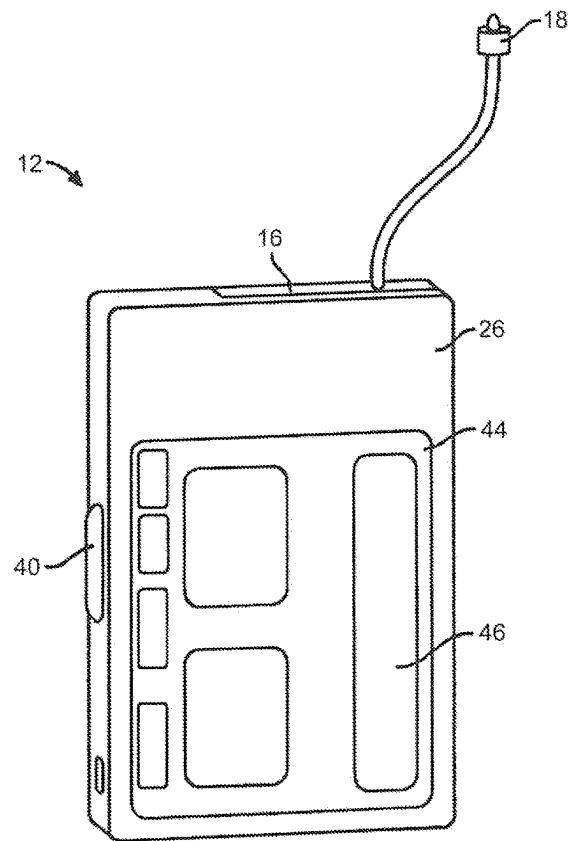


FIG. 1

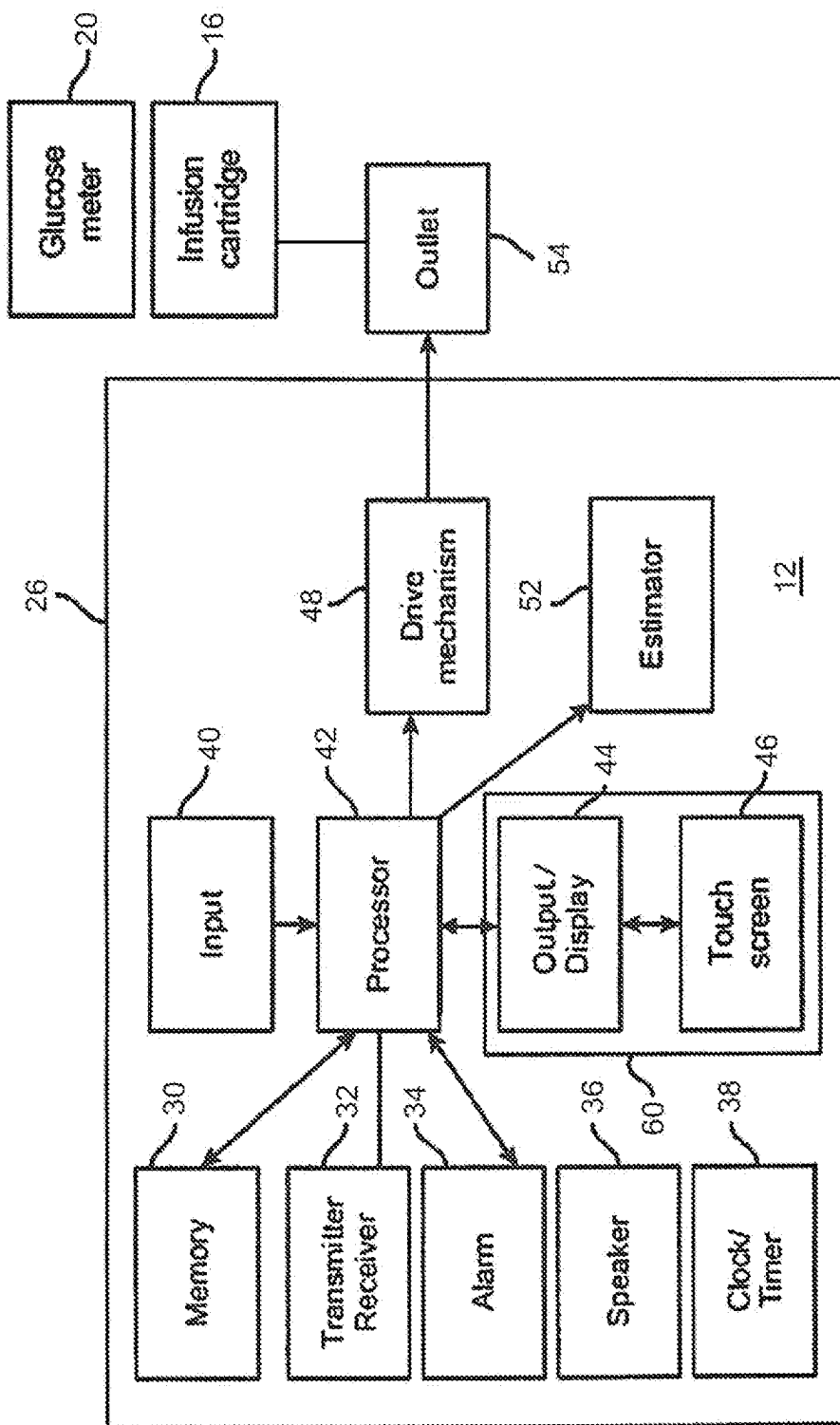


FIG. 2

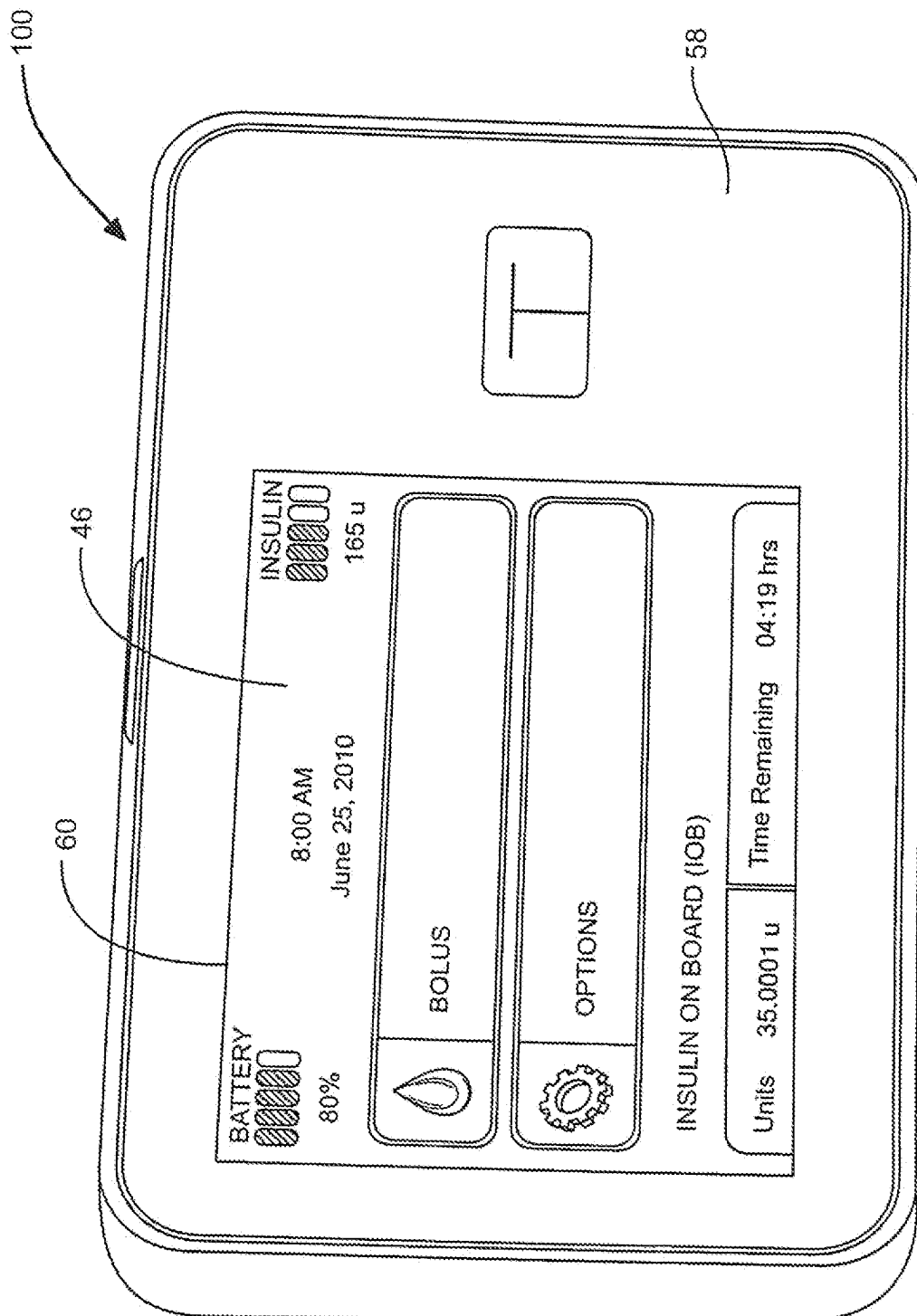


FIG. 3

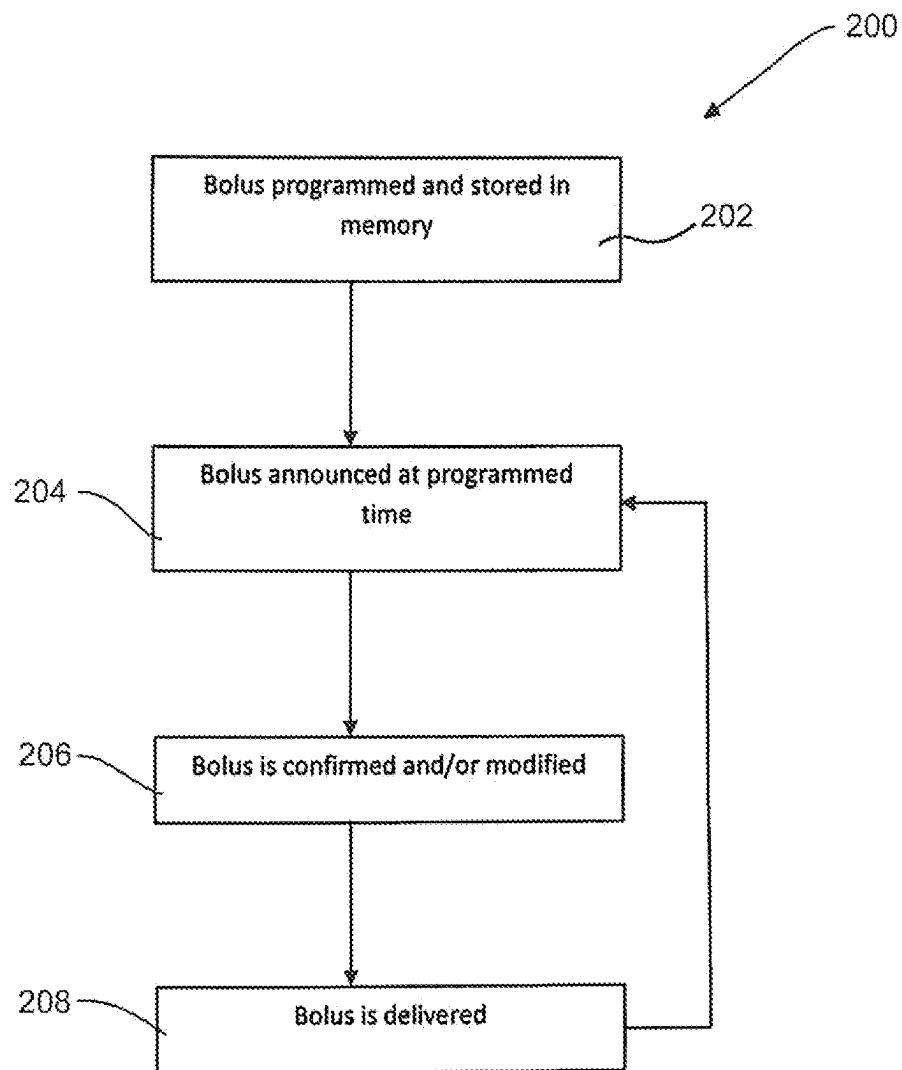


FIG. 4

1

SIMPLIFIED INSULIN DELIVERY FOR DIABETICS

RELATED APPLICATION

This application is a continuation of U.S. application Ser. No. 16/269,767 filed Feb. 7, 2019, which is a continuation of U.S. application Ser. No. 13/800,595 filed Mar. 13, 2013, now U.S. Pat. No. 10,201,656, which are hereby fully incorporated herein by reference.

FIELD OF THE INVENTION

The present invention relates to insulin pumps for the treatment of diabetes and, more particularly to a simplified insulin pump for use by Type II diabetics.

BACKGROUND

There are many applications in academic, industrial, and medical fields that benefit from devices and methods that are capable of accurately and controllably delivering fluids, such as liquids and gases that have a beneficial effect when administered in known and controlled quantities. Such devices and methods can be particularly useful in the medical field where treatments for many patients include the administration of a known amount of a substance at predetermined intervals.

Insulin-injecting pumps have been developed for the administration of insulin for those suffering from both type I and type II diabetes. Recently, continuous subcutaneous insulin injection and/or infusion therapy with portable infusion devices has been adapted for the treatment of diabetes. Such therapy may include the regular and/or continuous injection or infusion of insulin into the skin of a person suffering from diabetes, and offers an alternative to multiple daily injections of insulin by an insulin syringe or an insulin pen. Such pumps can be ambulatory/portable infusion pumps that are worn by the user and that may use replaceable cartridges. Examples of such pumps and various features that can be associated with such pumps include those disclosed in U.S. patent application Ser. No. 13/557,163, U.S. patent application Ser. No. 12/714,299, U.S. patent application Ser. No. 12/538,018, U.S. Provisional Patent Application No. 61/655,883, U.S. Provisional Patent Application No. 61/656,967 and U.S. Pat. No. 8,287,495, each of which is incorporated herein by reference.

Current infusion pumps typically allow for delivery of two infusion types—a basal rate and a bolus delivery. A basal rate typically delivers insulin at a constant rate over an extended period of time and is provided to maintain target glucose levels throughout the day when a user is not eating. Boluses are delivered to counteract carbohydrates consumed at meal times to maintain target glucose levels. For a meal bolus, for example, the user may enter the amount of carbohydrates the user is about to ingest and the user's carbohydrate ratio (the volume of insulin programmed to be delivered for every particular number of carbohydrates predicted to be consumed by the user). Based on this information, the infusion pump will generate an estimate of a bolus amount of insulin to be delivered. If accepted by the user, e.g., by entering a command such as by depressing a button, touching an object on a touchscreen, etc., the then-current basal delivery mode is suspended and the bolus delivery is initiated.

Careful management of insulin delivery in view of carbohydrate consumption, physical activity, and other factors

2

is critical in maintaining the health of type I diabetics. While delivery of insulin is equally important for the overall health of type II diabetics, the specific timing of the delivery and the precise matching of the amount of insulin to carbohydrates consumed is not as critical. In addition, type II diabetics are generally diagnosed only after years of not having to deal with monitoring carbohydrate intake, taking insulin injections, etc., as opposed to type I diabetics who are usually diagnosed at a much younger age. As a result of these and other factors, type II diabetics often do not have the motivation and/or the willingness to diligently monitor their carbohydrate intake, insulin levels, etc. as is required to properly use most insulin pumps.

Therefore, there is a need for an insulin pump and method that is simplified for the treatment of type II diabetics to provide a needed amount of insulin without requiring precise tracking and matching of carbohydrate intake with insulin delivered.

SUMMARY OF THE INVENTION

A simplified insulin pump allows type II diabetics to identify routine patterns in their daily lifestyle that provide a generally accurate typical pattern of activity and meals. An infusion regimen can be programmed into the pump that generally correlates with this typical pattern to provide a viable treatment option for a type II diabetic without the need for specific and precise matching of insulin to carbohydrate consumption or activity level.

In an embodiment, a portable insulin pump includes a delivery mechanism for delivering insulin to a user and a user interface to display operating parameters of the delivery mechanism and receive input from the user. A processor of the pump receives user input data defining a recurring meal bolus scheduled for delivery at a predetermined time on a type or category of day, such as a weekday. The processor provides an alert to the user through the user interface that a meal bolus is scheduled at the predetermined time on each day in the category of day and then causes the delivery mechanism to deliver the meal bolus to the user.

Certain embodiments are described further in the following description, examples, claims, and drawings. These embodiments will become more apparent from the following detailed description when taken in conjunction with the accompanying exemplary drawings.

BRIEF DESCRIPTION OF THE DRAWINGS

FIG. 1 is a perspective view of an infusion pump according to an embodiment of the present invention.

FIG. 2 is a block diagram representing an embodiment of an infusion pump.

FIG. 3 depicts a screen shot of a home screen page of a graphical user interface of an infusion pump according to an embodiment of the present invention.

FIG. 4 depicts a flowchart of a method of delivering a bolus of insulin according to an embodiment of the present invention.

DETAILED DESCRIPTION

Provided herein are systems, devices and methods for operating an infusion pump and particularly an insulin pump. Some embodiments may include advances in the internal components, the control circuitry, and improvements in a user interface of the systems and devices. The advances may allow for a safer and more accurate delivery

of medicament to a patient than is currently attainable today from other devices, systems, and methods. Although embodiments described herein may be discussed in the context of the controlled delivery of insulin, delivery of other medicaments, including, for example, glucagon, pramlintide, etc., as well as other applications are also contemplated. Device and method embodiments discussed herein may be used for pain medication, chemotherapy, iron chelation, immunoglobulin treatment, dextrose or saline IV delivery, or any other suitable indication or application. Non-medical applications are also contemplated.

FIG. 1 depicts an embodiment of a pump 12 such as an infusion pump that can include an internal pumping or delivery mechanism and reservoir for delivering medicament such as insulin to a patient and an output/display 44. The type of output/display 44 may vary as may be useful for a particular application. The type of visual output/display may include LCD displays, LED displays, plasma displays, OLED displays and the like. The output/display 44 may also be an interactive or touch sensitive screen 46 having an input device such as, for example, a touch screen comprising a capacitive screen or a resistive screen. The pump 12 may additionally include a keyboard or other input device known in the art for data entry, which may be separate from the display. The output/display 44 of the pump 12 may also include a capability to operatively couple to a secondary display device such as a laptop computer, mobile communication device such as a smartphone or personal digital assistant (PDA) or the like. Further details regarding such pump devices can be found in U.S. Patent Application No. 2011/0144586, which is incorporated herein by reference.

FIG. 2 illustrates a block diagram of some of the features that may be incorporated within the housing 26 of the pump 12. The pump 12 includes a processor 42 that functions to control the overall functions of the device. The infusion pump 12 may also include a memory device 30, a transmitter/receiver 32, an alarm 34, a speaker 36, a clock/timer 38, an input device 40, the processor 42, a user interface suitable for accepting input and commands from a user such as a caregiver or patient, a drive mechanism 48, and an estimator device 52. One embodiment of a user interface as shown in FIG. 2 is a graphical user interface (GUI) 60 having a touch sensitive screen 46 with input capability. The memory device 30 may be coupled to the processor 42 to receive and store input data and to communicate that data to the processor 42. The input data may include user input data and non-user/sensor input data. The input data from the memory device 30 may be used to generate therapeutic parameters for the infusion pump 12. The GUI 60 may be configured for displaying a request for the user to input data and for receiving user input data in response to the request, and communicating that data to the memory.

The processor 42 may communicate with and/or otherwise control the drive mechanism, output/display, memory, a transmitter/receiver and other components. In some embodiments, the processor 42 may communicate with a processor of another device, for example, a continuous glucose monitor (CGM), through the transmitter/receiver. The processor 42 may include programming that can be run to control the infusion of insulin or other medicament from the cartridge, the data to be displayed by the display, the data to be transmitted via the transmitter, etc. The processor 42 may also include programming that may allow the processor to receive signals and/or other data from an input device, such as a sensor that may sense pressure, temperature or other parameters. The processor 42 may determine the capacity of the drug delivery reservoir and/or the volume of

fluid disposed in the drug delivery reservoir and may set therapeutic parameters based on its determination.

The processor 42 may also include additional programming to allow the processor 42 to learn user preferences and/or user characteristics and/or user history data. This information can be utilized to implement changes in use, suggestions based on detected trends, such as, weight gain or loss. The processor can also include programming that allows the device to generate reports, such as reports based upon user history, compliance, trending, and/or other such data. Additionally, infusion pump device embodiments of the disclosure may include a “power off” or “suspend” function for suspending one or more functions of the device, such as, suspending a delivery protocol, and/or for powering off the device or the delivery mechanism thereof. For some embodiments, two or more processors may be used for controller functions of the infusion pump devices, including a high power controller and a low power controller used to maintain programming and pumping functions in low power mode, in order to save battery life.

The memory device 30 may be any type of memory capable of storing data and communicating that data to one or more other components of the device, such as the processor. The memory may be one or more of a Flash memory, SRAM, ROM, DRAM, RAM, EPROM and dynamic storage, for example. For instance, the memory may be coupled to the processor and configured to receive and store input data and/or store one or more template or generated delivery patterns. For example, the memory can be configured to store one or more personalized (e.g., user defined) delivery profiles, such as a profile based on a user’s selection and/or grouping of various input factors, past generated delivery profiles, recommended delivery profiles, one or more traditional delivery profiles, e.g., square wave, dual square wave, basal and bolus rate profiles, and/or the like. The memory can also store, for example, user information, history of use, glucose measurements, compliance and an accessible calendar of events.

The housing 26 of the pump 12 may be functionally associated with an interchangeable and a removable glucose meter 20 and/or infusion cartridge 16. The infusion cartridge 16 may have an outlet port 54 that may be connected to an infusion set (not shown) via an infusion set connector 18. Further details regarding some embodiments of various infusion pump devices can be found in U.S. Patent Application No. 2011/0144586, which is hereby incorporated by reference.

Referring to FIG. 3, a front view of the pump 12 is depicted. The pump 12 may include a user interface, such as, for example, a user-friendly GUI 60 on a front surface 58 or other convenient location of the pump 12. The GUI 60 may include a touch-sensitive screen 46 that may be configured for displaying data, facilitating data entry by a patient, providing visual tutorials, as well as other interface features that may be useful to the patient operating the pump 12.

In order to simplify the use of the pump 12 for Type II diabetic patients, the device can be designed so that insulin is delivered with minimal patient interaction. In one embodiment, boluses intended to cover carbohydrates consumed at meal times can be automatically delivered in fixed quantities at fixed times. The user therefore can receive the bolus insulin without having to count the carbohydrates in the meals or remember to enter the carbohydrates to initiate a bolus. As noted above, strict accuracy in carbohydrate consumption and corresponding insulin delivery is not generally required for Type II diabetics, and therefore such a simplified system can be employed.

5

FIG. 4 depicts a flowchart of a method of delivering a bolus of insulin 200 according to an embodiment of the present invention. Initially, a bolus is programmed into the pump 12 and saved into memory 30 at step 202. The bolus can be for a fixed amount, such as ten units of insulin, and at a fixed time or time period. The bolus can also be associated with a category of day on which it is to be given, such as on all weekdays or all Mondays. When the fixed time or the beginning of the time period is reached on an associated day, such as 8:00 a.m., the bolus can be announced to the user at step 204. The announcement can comprise an alert including one or more of an audio, visual or tactile indication. Alternatively, the bolus can automatically be delivered without reminding the user.

In some embodiments, the user can confirm the bolus at step 206. Additionally, the user may be able to modify the bolus. Such modifications can include, for example, a change in the volume of the bolus or a delay in delivery of the bolus. The user may also be able to cancel the bolus. If the user confirms the bolus, the bolus is delivered to the user at step 208 and the system reverts to step 204 the next time that the time fixed time or beginning of the time period occurs. Alternatively, the bolus may be automatically delivered to the user following the announcement unless the user positively acts to cancel or change the delivery of the bolus. Thus, rather than the user confirming the bolus to have it delivered, the user must affirmatively cancel the bolus to prevent it from being automatically delivered.

The pump may provide for multiple boluses of the same or different amounts at different times during any given day to be stored in memory. In one embodiment, three boluses are stored for each day, corresponding to an approximate breakfast time, lunch time, and dinner time. Boluses can be stored so that different amounts and/or timing of boluses can be set for weekdays and weekends or for each day of the week. A user may also be able to select a bolus schedule according to certain activities or conditions, such as when the user is sick or has a regularly occurring athletic activity. Thus, once the fixed bolus amounts and times are programmed into the device, no further user interaction with the device is required to determine the timing and amount of bolus deliveries regardless of the timing and size (i.e., number of carbohydrates) of the user's actual meals.

In some embodiments, each bolus can be delivered as an extended bolus. Thus, if the user expects to have lunch around noon on a given day, the bolus can, for example, be delivered as an extended bolus between the hours of 11:00 a.m. and 1:00 p.m. Such an extended bolus therefore helps account for variability in meal times while still being delivered with minimal-to-no user interaction. With such an extended bolus, the user also has the option of editing the bolus while it is being delivered to, for example, add insulin for an increased amount of carbohydrates than usual or decrease or cancel a remainder of the bolus if a smaller than usual meal is eaten. Alternatively, boluses may be delivered all at once at a specified meal time, such as noon. Further detail regarding bolus delivery that can be done with pumps as described herein can be found in U.S. patent application Ser. No. 13/800,387 "System and Method for Maximum Insulin Pump Bolus Override" by Walsh.

In another embodiment, the standard meal boluses delivered to the user can be incorporated into a basal rate. Thus, for users that already receive a continuous infusion of insulin as a basal rate, the bolus can be incorporated into that basal rate rather than being delivered in addition to the basal rate around meal times. The user's regular basal rate insulin needs and fixed meal bolus amounts can be combined to

6

calculate an adjusted basal rate that is increased to factor in the meal bolus amounts. Such a protocol ensures the user is receiving enough insulin while further reducing interactions the user must make with the device. As noted herein, such a system is feasible for type II diabetic users because the timing of delivery of insulin relative to meal times is not as critical as with type I users. With such a system, the pump need only be programmed initially and no further user interaction with the device is required until the insulin cartridge needs to be changed with a cartridge having a full insulin reservoir, or if the user desires to change the rate of the insulin being delivered. If the user wishes to modify the new basal rate for any reason, a temporary rate can be delivered.

With regard to the above detailed description, like reference numerals used therein may refer to like elements that may have the same or similar dimensions, materials, and configurations. While particular forms of embodiments have been illustrated and described, it will be apparent that various modifications can be made without departing from the spirit and scope of the embodiments herein. Accordingly, it is not intended that the invention be limited by the foregoing detailed description.

The entirety of each patent, patent application, publication, and document referenced herein is hereby incorporated by reference. Citation of the above patents, patent applications, publications and documents is not an admission that any of the foregoing is pertinent prior art, nor does it constitute any admission as to the contents or date of these documents.

Modifications may be made to the foregoing embodiments without departing from the basic aspects of the technology. Although the technology may have been described in substantial detail with reference to one or more specific embodiments, changes may be made to the embodiments specifically disclosed in this application, yet these modifications and improvements are within the scope and spirit of the technology. The technology illustratively described herein may suitably be practiced in the absence of any element(s) not specifically disclosed herein. The terms and expressions which have been employed are used as terms of description and not of limitation and use of such terms and expressions do not exclude any equivalents of the features shown and described or portions thereof and various modifications are possible within the scope of the technology claimed. Although the present technology has been specifically disclosed by representative embodiments and optional features, modification and variation of the concepts herein disclosed may be made, and such modifications and variations may be considered within the scope of this technology.

The invention claimed is:

1. An ambulatory infusion pump system comprising:
 - a reservoir configured to contain insulin;
 - a delivery mechanism configured to deliver insulin from the reservoir to a user; and
 - at least one processor configured to:
 - determine that a user will be eating a meal; and
 - cause the delivery mechanism to deliver a meal bolus of insulin of a single fixed amount to the user in a single continuous delivery operation in response to determining that the user will be eating the meal, wherein the single fixed amount of the meal bolus is predetermined without precisely matching the amount of the meal bolus with a number of carbohydrates to be consumed in the meal.

7

2. The system of claim 1, wherein the single fixed amount of insulin is a predetermined number of units of insulin.

3. The system of claim 2, further comprising a memory and wherein the predetermined number of units of insulin is saved into memory.

4. The system of claim 1, wherein the processor is configured to determine that a user will be eating a meal based on user input entered by the user.

5. The system of claim 1, where the processor is configured to automatically deliver the meal bolus of insulin after determining that the user will be eating a meal.

6. The system of claim 1, wherein the processor is further configured to receive a user confirmation of the meal bolus of insulin prior to delivering the meal bolus.

7. The system of claim 1, wherein the processor is further configured to receive a modification to the single fixed amount.

8. The system of claim 1, wherein the single fixed amount of insulin can vary based on a type of meal.

9. The system of claim 8, wherein the type of meal can include breakfast, lunch and dinner.

10. A method of providing insulin therapy to a user of an ambulatory infusion pump, the ambulatory infusion pump being part of an ambulatory infusion pump system, comprising:

determining with a processor of the ambulatory infusion pump system that a user will be eating a meal; and causing, with the processor, the ambulatory infusion pump to deliver a meal bolus of insulin of a single fixed

8

amount to the user in a single continuous delivery operation in response to determining with the processor of the ambulatory infusion pump system that the user will be eating the meal, wherein the single fixed amount of the meal bolus is predetermined without precisely matching the amount of the meal bolus with a number of carbohydrates to be consumed in the meal.

11. The method of claim 10, wherein the single fixed amount of insulin is a predetermined number of units of insulin.

12. The method of claim 11, further comprising storing the predetermined number of units of insulin in a memory.

13. The method of claim 10, wherein determining with the processor of the ambulatory infusion pump system that the user will be eating a meal includes receiving user input entered by the user.

14. The method of claim 10, wherein causing, with the processor, the ambulatory infusion pump to deliver the meal bolus includes automatically delivering the meal bolus of insulin after determining with the processor of the ambulatory infusion pump system that the user will be eating a meal.

15. The method of claim 10, further comprising receiving a user confirmation of the meal bolus of insulin prior to delivering the meal bolus.

16. The method of claim 10, further comprising receiving a modification to the single fixed amount.

* * * * *